Test 1:

READING PASSAGE 1

*You should spend about 20 minutes on **Questions 1–13**, which are based on Reading Passage 1 below.*

Think Small

Izon is a New Zealand company specialising in nanoscience – the study of atoms, molecules and other very small pieces of matter.

The new US headquarters of Izon are situated in Cambridge, Massachusetts, one of the most important global centres of scientific research and technology. Half the other offices in the building Izon shares have scientific names like Slegment, or the word 'pharmaceutical' in the name. In the past three years, Izon has expanded to 17 times its previous size. It's done so selling only one product that it's been developing since 2004, the qNano. This is an adjustable tool for the measurement and analysis of tiny particles in scientific research, which comes with a $27,000 price tag. Izon's success results from the fact that the qNano machine provides precise measurements, while similar machines of the same price only offer averaging techniques. In effect, researchers using the qNano are able to gather information about very small things, and it has many possible applications from drug delivery analysis through to environmental research.

Kristoffer Bolen is Izon's US head of sales. Alongside his full-time role with the company, he's just finished the fifth semester of six for his High-Tech Master of Business Administration at Northeastern University, which will be his second master's degree, alongside a Master of Science. These soon-to-be twin master's degrees provide Bolen with the perfect combination of skills for his job, which depends on him being able to manage the sales process and also comprehend in depth a very complicated piece of technology. When a customer buys a qNano device, an Izon representative will fly anywhere in America to train them how to use the instrument. The representative then checks in with the client on an ongoing basis to be completely informed about how each qNano is being used, and then liaises with technicians back in New Zealand.

The Izon story started 8 years ago in New Zealand. Hans van der Voorn was aimless, having sold his own company, and was looking for something more taxing to set his mind to. Van der Voorn was introduced to three scientists with an idea that involved simply a small plastic membrane with a tiny adjustable hole to measure a range of very small particles. Initially he put up an investment of $1.5 million to move the process forward to the development of a prototype, and this led to the founding of Izon and the production of the qNano. Van der Voorn believes that the worlds of science and business sometimes don't understand each other very well. However, he thinks that engineering can be a bridge between the two, and his long experience in that field has been crucial to the company's success. He soon realised that Izon was going to need people on the ground overseas if it was going to grow,

so the US office was opened. For now Izon trades in the US as a foreign company, so avoids the high level of compliance that comes with being an American-registered company. But he sees that one day soon this will change, as Izon's current position disallows it from entering into partnerships locally.

Marketing the qNano is complicated. Izon needs scientists both to buy and advertise the product. The science world may seem like a big playing field, but it's actually a small group. As a result, the critical part of the company's marketing is articles that clients publish describing work they have performed with the qNano. So for Izon the sale isn't enough in itself; it also has a stake in how customers are using the qNano. That's where Yaniv Gaynor comes in. As development and training manager, he's involved throughout the sales process and manages most of the customer's post-sale experience. The sales process is new for Gaynor, who shifted cities and switched from a role as teacher at the University of Minnesota to his first private sector role with Izon. 'It's tricky, it's challenging, and it's not in my comfort zone,' he concedes. But nine out of ten times, if there's an unhappy customer, he's able to turn their experience around. And there's another part to this, because sitting with customers as they learn about the qNano, Gaynor gets to see how they are using the product. 'People ask very good questions, and these are great triggers for future development,' he says.

'Our business is selling research tools,' van der Voorn says. But he knows that the application range of this tool is wide. For instance, Izon is engaged in a project with Harvard University, looking for a particular type of particle of blood that is a symptom of the medical condition of thrombosis. If in the future they can identify this particle, they can design a simple test for it. This would make it easier for doctors to diagnose thrombosis. Another planned development is eventually to shift the US base to more spacious offices on the other side of town. With larger headquarters, the company will be able to bring clients into the Izon building for training with its product, rather than travelling to them, which will hopefully result in an even closer relationship between the company and its customers.

Questions 1–7

Do the following statements agree with the information given in Reading Passage 1?

*In boxes **1–7** on your answer sheet, write:*

TRUE	*if the statement agrees with the information*
FALSE	*if the statement contradicts the information*
NOT GIVEN	*if there is no information on this*

1. Izon's new US headquarters are located in an area popular with other science businesses.

2. Izon's growth in the last three years has been faster than the company predicted.

3. Since 2004, Izon has developed a range of equipment for use in scientific research.

4. One advantage of the qNano is that it is cheaper than its competitors.

5. Drug delivery analysis will be the most profitable application of the qNano.

6. Kristoffer Bolen is still completing his studies.

7. Bolen's job requires him to fully understand how the qNano works.

TRUE IELTS

Questions 8–13

Complete the notes below.

*Choose **ONE WORD ONLY** from the passage for each answer.*

*Write your answers in boxes **8–13** on your answer sheet.*

The Izon Story

Hans van der Voorn

- He made the **8** _____ which enabled the prototype to be created.
- His background in **9** _____ as well as business gave him the skills to run Izon.

Izon in the United States

- It is unable to form partnerships in the US because it's not registered there.
- The company's marketing relies on **10** _____ about its customers' research.
- Yaniv Gaynor was formerly a **11** _____, with no experience in sales.
- Development of the product is influenced by customers' questions.

Plans for the future

- To diagnose thrombosis, a **12** _____ may be developed.
- **13** _____ will be done at Izon's US headquarters.

ĐÁP ÁN:

1. **TRUE**

   (Trụ sở mới của Izon nằm ở Cambridge, Massachusetts, nơi có nhiều văn phòng khác mang tên khoa học hoặc dược phẩm .
2. **NOT GIVEN**

   (Văn bản nói Izon đã mở rộng quy mô gấp 17 lần, nhưng không đề cập việc này nhanh hơn hay chậm hơn dự kiến của công ty .
3. **FALSE**

   (Từ năm 2004, Izon chỉ tập trung phát triển và bán duy nhất một sản phẩm là qNano).
4. **FALSE**

   (Văn bản so sánh qNano với các máy khác có cùng mức giá, không nói nó rẻ hơn).
5. **NOT GIVEN**

   (Có nhắc đến ứng dụng phân tích phân phối thuốc, nhưng không khẳng định đây là ứng dụng mang lại lợi nhuận cao nhất).
6. **TRUE**

   (Kristoffer Bolen vừa kết thúc học kỳ thứ 5 trên tổng số 6 học kỳ của chương trình Thạc sĩ).
7. **TRUE**

   (Công việc của Bolen đòi hỏi anh phải hiểu sâu về loại công nghệ rất phức tạp này).
8. **investment**

   (Hans van der Voorn đã bỏ ra khoản đầu tư 1,5 triệu đô la để phát triển nguyên mẫu).
9. **engineering**

   (Kinh nghiệm lâu năm trong lĩnh vực kỹ thuật là yếu tố quan trọng giúp ông điều hành Izon).
10. **articles**

    (Tiếp thị của công ty dựa trên các bài báo mà khách hàng xuất bản về nghiên cứu của họ).
11. **teacher**

    (Yaniv Gaynor trước đây là một giáo viên tại Đại học Minnesota).

12. **test**

    (Một bài kiểm tra đơn giản có thể được phát triển để giúp bác sĩ chẩn đoán bệnh huyết khối).

13. **training**

    (Trong tương lai, việc đào tạo khách hàng sẽ được thực hiện trực tiếp tại trụ sở của Izon thay vì phải di chuyển đến chỗ khách).

TRUE IELTS

READING PASSAGE 2

*You should spend about 20 minutes on **Questions 14–26**, which are based on Reading Passage 2 below.*

Will Eating Less Make You Live Longer?

The latest in a conflicting series of studies suggests calorie restriction could potentially slow ageing in humans

A Calorie restriction, or 'semi-starvation' as some refer to it, has been proven to extend lifespan in many living organisms from yeast to mice, but the picture for primates, including humans, is not so clear. Research published by a team at the University of Wisconsin in the United States shows that rhesus monkeys also live longer on a calorie-restricted diet. But those findings disagree with research by the National Institute on Aging (NIA) in Maryland, also in the United States. Rozalyn Anderson, of the Wisconsin team, says the research is not intended as a recommendation of calorie restriction. 'I find the idea monumentally unattractive!' she says. 'We study it because it is so effective at delaying ageing and the onset of age-related disease. It's a way to tease out what it is that creates increased disease vulnerability as a function of age.'

B Both groups started long-term trials on rhesus monkeys in the late 1980s to determine whether calorie restriction would extend the lifespan of primates. In mice, many experiments had come to the same finding: feed them a diet with 30% fewer calories and see a lifespan extension of 40%. The monkey trials were set up in a similar way: researchers took the calorie content of a standard monkey diet, cut it by 30% (while continuing to supply all essential nutrients) and monitored whether those monkeys lived longer, healthier lives than those on the standard diet.

C In 2009, with the monkeys approaching old age, preliminary findings of the two trials started coming in. For the Wisconsin monkeys, calorie restriction seemed to be working. Compared to well-fed control animals, the lean monkeys were living longer and suffering less from the diseases of ageing: diabetes, heart disease and brain diseases. However, in 2012 the NIA results emerged with a dramatically different conclusion: their monkeys were not living any longer than the controls, although they were healthier.

D The Wisconsin group's latest results confirm that their calorie-restricted monkeys are living longer than the controls. They also offer a possible explanation of why the two groups' findings don't agree, and that lies in the treatment of the control group. The Wisconsin study began with monkeys in early adulthood. Initially, all the monkeys were allowed to eat as much as they liked. A few months into the trial, the monkeys were placed into one of two groups: the controls (who continued to be fed as much as they wished) and the calorie-restricted monkeys (who were given an individualised diet of 30% less than they were previously eating).

E The NIA study differed in two ways. First, the control group of monkeys were not allowed to eat as much as they wished. They were given a diet considered to represent a normal calorie count, while the calorie-restricted monkeys were fed 30% less than that. Second,

whereas the Wisconsin monkeys were given highly processed food high in sucrose, making it easy to standardise, the NIA diet was based on whole grains, fish oils, and was very low in sugar. These different settings for the normal control diet may provide an explanation of why the two groups showed different results.

F The Wisconsin group may in effect have studied the effects of 'overeating'. Their control animals weighed up to 10% more than average for their age and gender. Compared to them, the calorie-restricted animals not only suffered fewer diseases, they lived longer. Julie Mattison, head of the NIA study, observes that an overweight person who goes to a fast food restaurant every day will obviously benefit if their calories are cut back by 30%.

G The NIA study fed their control monkeys what they considered a 'standard' caloric intake and saw that longevity for monkeys that were fed 30% less was the same. But in Anderson's view, the NIA control monkeys were not fed enough; the NIA was calorie restricting both groups of animals. Their older female control monkeys, for example, weighed nearly 20% less than the national average. Indeed, Anderson points out that several of the NIA control monkeys have lived past the age of 40, far exceeding the 27-year average lifespan for captive rhesus monkeys. 'That's the maximum lifespan ever detected for the species, so the idea that their intervention is doing nothing is really at odds with the data,' says Anderson.

H However, neither group had the right notion of a standard diet according to Leonie Heilbronn, who researches calorie restriction and healthy ageing at the University of Adelaide in Australia. Heilbronn points out that the NIA controls were a very healthy set of monkeys and leaner than a control monkey should be. On the other hand, she notes, the Wisconsin controls were a little bigger than they had to be. On balance, Heilbronn agrees with the Wisconsin researcher's argument. 'I think these studies suggest calorie restriction definitely will increase lifespan,' she says.

I Both studies agree that cutting calories is beneficial to health — in both cases the calorie-restricted monkeys had fewer diseases related to ageing. As to the question of whether calorie restriction extends lifespan in primates, Anderson and Mattison are currently working together to compare the two studies' raw data. They plan to co-publish a joint analysis. The current data conflict will ultimately provide deeper insights into ageing, Anderson predicts. 'The fact the two studies were set up differently, asking the same question in different ways — I think we will gain maximally from that.'

Questions 14–19

Reading Passage 2 has nine paragraphs, ***A–I****.*

Which paragraph contains the following information?

Write the correct letter, ***A–I****, in boxes 14–19 on your answer sheet.*

14 a reference to studies based on the same level of calorie reduction with different animals

15 a comment that some control monkeys in one study reached an older age than normal for animals of their kind

16 distinctions between the types of food in each study that may have led to a contrast in findings

17 examples of health problems which monkeys on calorie-restricted diets were less likely to get

18 a researcher's negative opinion of a calorie-restricted diet

19 a reference to the stage in the monkeys' lives at which research commenced in one study

TRUE IELTS

Questions 20–23

Look at the following statements (Questions 20–23) and the list of researchers below.

Match each statement with the correct researcher, ***A, B or C****.*

Write the correct letter, ***A, B or C****, in boxes 20–23 on your answer sheet.*

NB *You may use any letter more than once.*

20 It is good that the two studies took dissimilar approaches.

21 Neither study used diets that are typical for monkeys.

22 Calorie restriction is a method of finding out about health issues connected with ageing.

23 Calorie reduction will have a positive effect on people who have unhealthy diets.

List of Researchers

A Rozalyn Anderson
B Julie Mattison
C Leonie Heilbronn

TRUE IELTS

Questions 24–26

Complete the summary below.

Choose ***ONE WORD ONLY*** *from the passage for each answer.*

Write your answers in boxes 24–26 on your answer sheet.

Agreement between the two studies on calorie restriction in rhesus monkeys

The two studies were in agreement that it is beneficial to cut calories, in terms of health. Monkeys whose calories were restricted suffered fewer diseases that are associated with the **24** _____ process. Anderson and Mattison have undertaken research to determine if a calorie-restricted diet increases primates' **25** _____. Although there is a **26** _____ between the two sets of data, Anderson believes they will help researchers reach a better understanding.

ĐÁP ÁN:

14. B (Đề cập đến việc cắt giảm 30% calo được thực hiện trên cả chuột và khỉ).

Đoạn chứa đáp án:

In mice, many experiments had come to the same finding: feed them a diet with 30% fewer calories and see a lifespan extension of 40%. The monkey trials were set up in a similar way: researchers took the calorie content of a standard monkey diet, cut it by 30%.

15. G (Đề cập đến việc một số con khỉ trong nhóm đối chứng của NIA sống quá 40 tuổi, vượt xa mức trung bình).

Đoạn chứa đáp án:

Their older female control monkeys, for example, weighed nearly 20% less than the national average. Indeed, Anderson points out that several of the NIA control monkeys have lived past the age of 40, far exceeding the 27- year average lifespan for captive rhesus monkeys.

16. E (Phân biệt giữa thức ăn chế biến sẵn nhiều đường của Wisconsin và ngũ cốc nguyên hạt của NIA).

Đoạn chứa đáp án:

These different settings for the normal control diet may provide an explanation of why the two groups showed different results.

17. C (Liệt kê các bệnh như tiểu đường, bệnh tim và bệnh não).

Đoạn chứa đáp án:

Compared to well-fed control animals, the lean monkeys were living longer and suffering less from the diseases of ageing: diabetes, heart disease and brain diseases.

18. A (Rozalyn Anderson nói rằng bà thấy ý tưởng ăn kiêng này "cực kỳ không hấp dẫn").

Đoạn chứa đáp án:

‘I find the idea monumentally unattractive!’ she says. ‘We study it because it is so effective at delaying ageing and the onset of age-related disease. It’s a way to tease out what it is that creates increased disease vulnerability as a function of age.’

19. D (Đề cập đến việc nghiên cứu của Wisconsin bắt đầu khi khỉ ở giai đoạn đầu trưởng thành).

Đoạn chứa đáp án:

A few months into the trial, the monkeys were placed into one of two groups: the controls (who continued to be fed as much as they wished) and the calorie-restricted monkeys (who were given an individualised diet of 30% less than they were previously eating).

20. A (Rozalyn Anderson: "...I think we will gain maximally from that.")

21. C (Leonie Heilbronn: "...neither group had the right notion of a standard diet...")

22.A (Rozalyn Anderson: "It's a way to tease out what it is that creates increased disease vulnerability...")

23.B (Julie Mattison: "...an overweight person who goes to a fast food restaurant every day will obviously benefit...")

24. ageing (hoặc **aging**) (Các bệnh liên quan đến quá trình lão hóa).

Đoạn chứa đáp án:

in both cases the calorie-restricted monkeys had fewer diseases related to ageing

25. lifespan (Nghiên cứu xem liệu chế độ ăn này có làm tăng tuổi thọ của linh trưởng không).

Đoạn chứa đáp án:

As to the question of whether calorie restriction extends lifespan in primates

26. conflict (Mặc dù có sự mâu thuẫn giữa hai tập dữ liệu)

Đoạn chứa đáp án:

The current data conflict will ultimately provide deeper insights into ageing, Anderson predicts. 'The fact the two studies were set up differently, asking the same question in different ways — I think we will gain maximally from that.'

READING PASSAGE 3

*You should spend about 20 minutes on **Questions 27–40**, which are based on Reading Passage 3 below.*

When people are ‘deaf’ to music

Music has long been considered a uniquely human concept. In fact, most psychologists agree that music is a universal human instinct. Like any ability, however, there is great variation in people’s musical competence. For every brilliant pianist in the world, there are several people we refer to as ‘tone deaf’. It is not simply that people with tone deafness (or ‘amusia’) are unable to sing in tune, they are also unable to discriminate between tones or recognize familiar melodies. Such a ‘disorder’ can occur after some sort of brain damage, but recently research has been undertaken in an attempt to discover the cause of congenital amusia (when people are born with the condition), which is not associated with any brain damage, hearing problems, or lack of exposure to music.

According to the research of Dr. Isabelle Peretz of the University of Montreal, amusia is more complicated than the inability to distinguish pitches. An amusic (a person who has the condition of amusia) can distinguish between two pitches that are far apart, but cannot tell the difference between intervals smaller than a half step on the Western diatonic scale, while most people can easily distinguish differences smaller than that. When listening to melodies which have had a single note altered so that it is out of key with the rest of the melody, they do not notice a problem. As would be expected, amusics perform significantly worse at singing and tapping a rhythm along with a melody than do non-amusics.

The most fascinating aspect of amusia is how specific to music it is. Because of music’s close ties to language, it might be expected that a musical impairment may be caused by a language impairment. Studies suggest, however, that language and music ability are independent of one another. People with brain damage in areas critical to language are often still able to sing, despite being unable to communicate through speech. Moreover, while amusics show deficiencies in their recognition of pitch differences in melodies, they show no such deficiencies in tonal languages. Amusics who speak tonal languages, such as Chinese, do not report having any difficulty discriminating between words that differ only in their intonation. The linguistic cues inherent in speech make discrimination of meaning much easier for amusics. Amusics are also successful most of the time at detecting the mood of a melody, can identify a speaker based on his or her voice and can discriminate and identify environmental sounds.

Recent work has been focused on locating the part of the brain that is responsible for amusia. The temporal lobes of the brain, the location of the primary auditory cortex, have been considered. It has long been believed that the temporal lobes, especially the right temporal lobe, are most active when engaged in musical activity, so any musical disability should logically stem from here as well. Because it has been shown that there is no hearing deficit in amusia, researchers moved on to the temporal neocortex, which is where more sophisticated processing of musical cues was thought to take place. New studies, however, have suggested that the deficits in amusics are located outside the auditory cortex. Brain scans of amusics do not show any reaction at all to differences smaller than a half step.

When changes in tones are large, their brains overreact, showing twice as much activity on the right side of the brain as a normal brain hearing the same thing. These differences do not occur in the auditory cortex, indicating again that the deficits of amusia lie not in hearing impairment, but in higher processing of melodies.

So what does this all mean? Looking only at the research of Peretz in the field of neuropsychology of music, it would appear that amusia is some sort of disorder. As a student of neurobiology, however, I am skeptical. Certainly the studies by Peretz that have found significant differences between the brains of so-called amusics and normal brains are legitimate. The more important question now becomes one of normality. Every trait from skin color to intelligence to mood exists on a continuum—there is a great deal of variation from one extreme to the other. Just because we recognize that basic musical ability is something that the vast majority of people have, this doesn't mean that the lack of it is abnormal.

What makes an amusic worse off than a musical prodigy? Musical ability is culturally valued, and may have been a factor in survival at one point in human history, but it does not seem likely that it is being selected for on an evolutionary scale any longer. Darwin believed that music was adaptive as a way of finding a mate, but who needs to be able to sing to find a partner in an age when it is possible to express your emotions through a song on your iPod?

While the idea of amusia is interesting, it seems to be just one end of the continuum of innate musical ability. Comparing this 'disorder' to learning disorders like a specific language impairment seems to be going too far. Before amusia can be declared a disability, further research must be done to determine whether lack of musical ability is actually detrimental in any way. If no disadvantages can be found of having amusia, then it is no more a disability than having poor fashion sense or bad handwriting.

TRUE IELTS

Questions 27–31

Choose the correct letter, ***A, B, C or D****.*

Write the correct letter in boxes 27–31 on your answer sheet.

27 What does the writer tell us about people with tone deafness (amusia) in the first paragraph?

A They usually have hearing problems
B Some can play a musical instrument very well
C Some may be able to sing well-known melodies
D They have several inabilities in regard to music

28 What is the writer doing in the second paragraph?

A outlining some of the factors that cause amusia
B summarising some findings about people with amusia
C suggesting that people with amusia are disadvantaged
D comparing the singing ability of amusics with their sense

29 What does the writer say about the relationship between language ability and musical ability?

A People who are unable to speak can sometimes sing
B People with amusia usually have language problems too
C Speakers of tonal languages like Chinese rarely have amusia
D People with amusia have difficulty recognizing people by their voices

30 In the third paragraph, the writer notes that most amusics are able to

A learn how to sing in tune
B identify a song by its tune
C distinguish a sad tone from a happy tune
D recognise when a singer is not singing in tune

31 What is the writer doing in the fourth paragraph?

A claiming that amusics have problems in the auditory cortex
B outlining progress in understanding the brains of amusics
C proving that amusia is located in the temporal lobes
D explaining why studies of hearing are difficult

Questions 32–36

Do the following statements agree with the views of the writer in Reading Passage 3?

In boxes 32–36 on your answer sheet, write

YES — if the statement agrees with the views of the writer
NO — if the statement contradicts the views of the writer
NOT GIVEN — if it is impossible to say what the writer thinks about this

32 Peretz's research suggesting that amusia is a disorder is convincing.

33 People with musical ability are happier than those without this ability.

34 It is inappropriate to consider amusia as a real disorder.

35 People with amusia often have bad handwriting.

Questions 36–40

*Complete each sentence with the correct ending, **A–H**, below.*

*Write the correct letter, **A–H**, in boxes 36–40 on your answer sheet.*

36 The reason why some people are born with amusia is

37 One of the difficulties amusics experience is

38 For amusics, discrimination of meaning in speech is

39 Certain reactions in the brain of an amusic are

40 In most cultures, musical ability is

A an inability to hear when spoken language rises and falls.
B considered to be desirable.
C an inability to follow the beat of music.
D not a problem.
E not yet well understood.
F a result of injury to the mother.
G more marked than with other people.
H associated with intelligence.

ĐÁP ÁN:

27. D (They have several inabilities in regard to music)

Giải thích: Đoạn 1 nêu rõ người bị amusia không chỉ không thể hát đúng nhạc điệu mà còn không thể phân biệt các tông nhạc hoặc nhận ra các giai điệu quen thuộc.
The first paragraph states they are "unable to sing in tune," "unable to discriminate between tones or recognize familiar melodies." This lists several inabilities.

28. B (summarising some findings about people with amusia)

Giải thích: Đoạn 2 tóm tắt các phát hiện của Tiến sĩ Isabelle Peretz về khả năng phân biệt cao độ, sự nhạy cảm với các nốt bị thay đổi và kỹ năng gõ nhịp của người bệnh.
The second paragraph details specific findings from Dr. Peretz's research, such as the size of intervals amusics can and cannot distinguish and their performance on rhythm tasks.

29. A (People who are unable to speak can sometimes sing)

Giải thích: Đoạn 3 cho biết những người bị tổn thương não ở các khu vực quan trọng về ngôn ngữ thường vẫn có thể hát, dù không thể giao tiếp bằng lời nói.
The third paragraph states, "People with brain damage in areas critical to language are often still able to sing, despite being unable to communicate through speech."

30. C (distinguish a sad tone from a happy tune)

Giải thích: Bài viết đề cập rằng người bị amusia hầu hết thời gian vẫn thành công trong việc nhận ra tâm trạng (mood) của một giai điệu.
The third paragraph states that amusics "are also successful most of the time at detecting the mood of a melody".

31. B (outlining progress in understanding the brains of amusics)

Giải thích: Đoạn 4 mô tả quá trình nghiên cứu từ việc xem xét thùy thái dương đến vỏ não thái dương và các phát hiện mới về vị trí khiếm khuyết ngoài vỏ não thính giác.

The third paragraph states that amusics "are also successful most of the time at detecting the mood of a melody".

32. NO

Giải thích: Tác giả bày tỏ sự hoài nghi (skeptical) đối với quan điểm coi amusia là một chứng rối loạn (disorder).
In the fifth paragraph, the writer states, "As a student of neurobiology, however, I am skeptical" about the idea that amusia is a disorder. This shows disagreement.

33. NOT GIVEN

Giải thích: Bài đọc không so sánh mức độ hạnh phúc giữa người có và không có khả năng âm nhạc.
The passage discusses musical ability as a trait but makes no mention of the relative happiness of people with or without it.

34. YES

Giải thích: Tác giả cho rằng việc so sánh amusia với các chứng rối loạn học tập là "quá xa vời" (going too far) và cần nghiên cứu thêm trước khi tuyên bố nó là một khiếm khuyết.
n the final paragraph, the writer argues that if amusia has no detrimental disadvantages, "then it is no more a disability than having poor fashion sense or bad handwriting." This supports the idea that it is inappropriate to consider it a real disorder.

35. NOT GIVEN

Giải thích: "Chữ viết tay xấu" chỉ là một ví dụ so sánh (analogy) của tác giả ở cuối bài, không phải là một khẳng định rằng người bị amusia thường có đặc điểm này.
The final sentence is a hypothetical comparison: "it is no more a disability than... bad handwriting." The writer is not stating that people with amusia actually have bad handwriting.

36. E (not yet well understood): Lý do tại sao một số người sinh ra đã bị amusia hiện vẫn đang được nghiên cứu để tìm nguyên nhân.
The first paragraph states that "research has been undertaken in an attempt to discover the cause of congenital amusia," implying the cause is not yet known.

37. C (an inability to follow the beat of music): Một khó khăn là họ thực hiện tệ hơn đáng kể trong việc gõ nhịp theo giai điệu.
The second paragraph states that amusics "perform significantly worse at... tapping a rhythm along with a melody".

38. D (not a problem): Đối với họ, việc phân biệt ý nghĩa trong lời nói không phải là vấn đề vì có các manh mối ngôn ngữ hỗ trợ.
The third paragraph states that amusics, who have trouble with pitch in melodies, "do not report having any difficulty discriminating between words that differ only in their intonation" in tonal languages.

39. G (more marked than with other people): Một số phản ứng trong não họ (khi thay đổi tông nhạc lớn) mạnh gấp đôi so với não người bình thường.
The fourth paragraph says when tonal changes are large, amusics' brains "overreact, showing twice as much activity on the right side of the brain as a normal brain."

40. B (considered to be desirable): Trong hầu hết các nền văn hóa, khả năng âm nhạc được coi trọng (culturally valued).

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